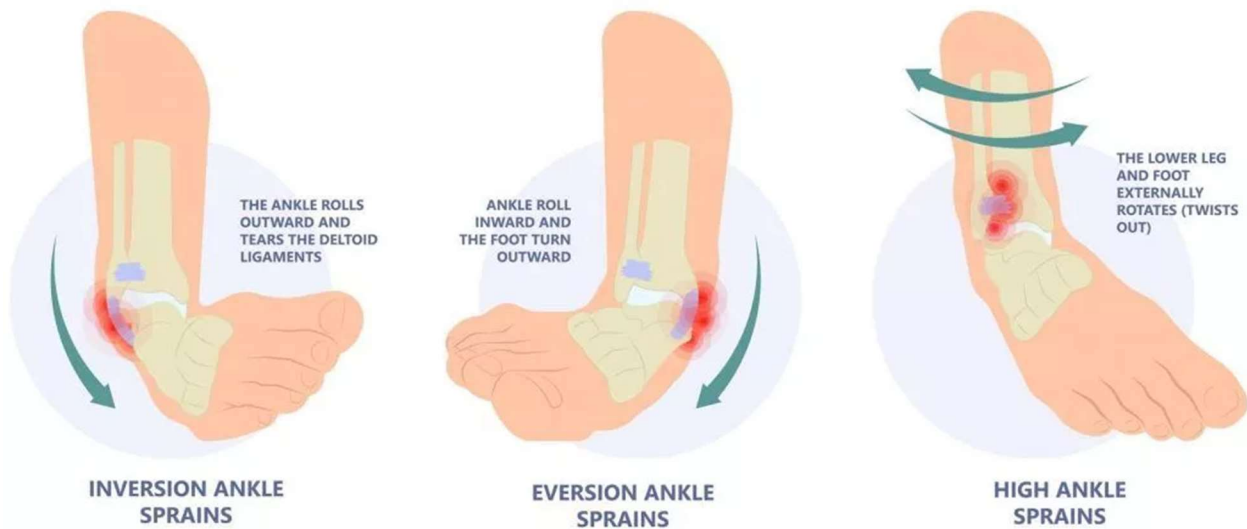


TYPES OF ANKLE SPRAINS



A sprain is the most common ankle injury. It is usually caused by a twist to the ankle and can be treated using the RICE procedure:

- **Rest** the affected part.
- **Cool** the injury with Ice.
- **Provide** comfortable Compression support with elastic bandaging.
- **Elevate** the injury.

If the casualty cannot bear any weight on the injured leg or there is severe pain, swelling, and/or deformity at the ankle, suspect a break and treat it as a fracture of the lower leg near the ankle. Be aware too, however, that a casualty may have a fracture and still be able to walk and move his toes. If you are in any doubt about an ankle injury, treat it as a fracture.

What to do.

1. Support the ankle in the most comfortable position for the casualty, preferably raised.
2. Apply a cold compress, such as an ice pack or a cold pad, to the site to reduce swelling and bruising.

3. Apply comfortable support to the ankle. Leave the cold compress in place or wrap a layer of soft padding around the area. Bandage the ankle with a support bandage that extends from the base of the foot to the upper extent of the pain (below).



4. Raise and support the injured limb. Check the circulation beyond the bandaging every ten minutes. If the circulation is impaired, loosen the bandage. If you suspect a broken bone, arrange to send the casualty to the hospital.

SHOULDER INJURY

A fall onto the shoulder or an outstretched arm or a wrenching force may pull the head of the upper arm bone (humerus) out of the joint socket—dislocation of the shoulder. At the same time, ligaments around the shoulder joint may be torn. This injury can be very painful. Some people experience repeated dislocations and may need a strengthening operation on the affected shoulder. A fall onto the point of the shoulder may damage the ligaments that brace the collarbone at the shoulder. Other shoulder injuries include damage to the joint capsule and to the tendons around the shoulder; these injuries tend to be common in older people. To treat a shoulder sprain, rest the affected part, cool the injury with ice, and use a sling to provide comfortable support.

Recognition

There may be:

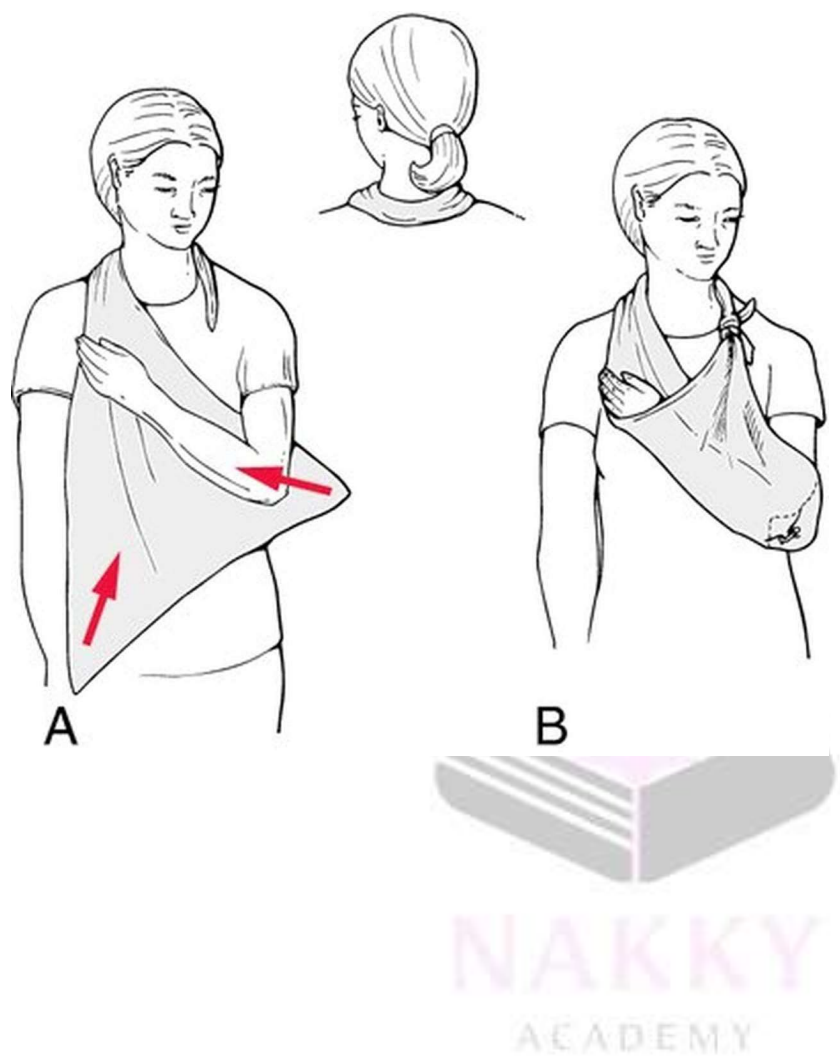
- Severe pain, increased by movement; the pain may make the casualty reluctant to move.
- Attempts by the casualty to relieve pain by supporting the arm and inclining the head to the injured side.
- A flat, angular look to the shoulder.

Your Aim

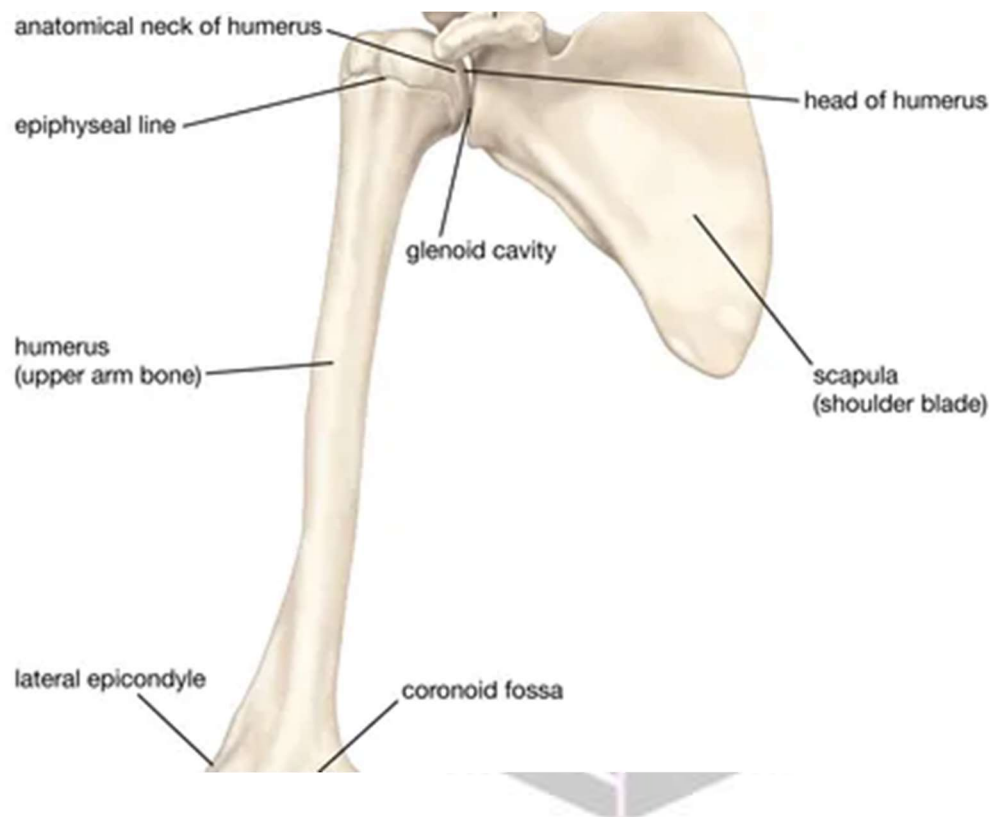
- To support and immobilize the injured limb.
- To arrange transportation to the hospital.

What to do.

1. Help the casualty sit down. Gently place the arm on the injured side across her body in the position that is most comfortable. Ask the casualty to support her elbow on the injured side, or help her do it.
2. Support the arm on the injured side with an arm sling.
3. For extra support if necessary, secure the arm to the chest by tying a broadfold bandage around the chest and the sling.
4. Arrange to take or send the casualty to the hospital in the position she finds most comfortable.



UPPER ARM INJURY



A serious form of upper arm injury is a ***fracture of the long bone in the upper arm (humerus)***. The bone may be fractured across the center by a direct blow. However, it is much more common, especially in elderly people, for the arm bone to break at the shoulder end, usually in a fall. A fracture at the top of the bone is usually a stable injury because the broken bone ends stay in place. For this reason, it may not be immediately apparent that the bone is broken, although the arm is likely to be painful. There is a possibility that the casualty will cope with the pain and leave the fracture untreated for some time.

Recognition

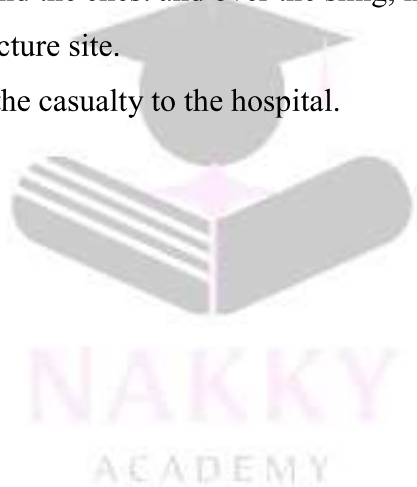
There may be:

- Pain, increased by movement.
- Tenderness and deformity over the site of a fracture.
- Rapid swelling.

- Bruising, which may develop more slowly.

What to do

1. Help the casualty sit down. Remove all jewelry such as bracelets, rings, and watches. Gently place the forearm horizontally across her body in the position that is most comfortable. Ask her to support her elbow if possible.
2. Slide a triangular bandage in position between the arm and the chest, ready to make an arm sling (p.39). Place soft padding between the injured arm and the body, then support the arm and its padding in an arm sling.
3. For extra support, or if the trip to the hospital is prolonged, secure the arm by tying a broad-fold bandage around the chest and over the sling; make sure that the broad-fold bandage is below the fracture site.
4. Arrange to take or send the casualty to the hospital.



MODULE SUMMARY

- 1. Medical emergencies.**
- 2. Monitoring vital signs.**
- 3. Conducting CPR- Unconscious adult.**
- 4. Conducting CPR- Child.**
- 5. Conducting CPR- Infant.**
- 6. Management of Falls & Fractures.**
- 7. Hemorrhage.**
- 8. Saving a Choking Adult.**
- 9. Saving a Choking Child & Infant.**
- 10. Fit and/or epileptic Seizures (convulsions).**
- 11. Saving a drowning casualty.**
- 12. Managing Asthma attacks.**
- 13. Fires & Burns.**
- 14. Spinal injury.**
- 15. Ankle injury.**
- 16. Shoulder injury.**
- 17. Upper arm Injury.**

